

Lothian Primary Care CA125 Guidance

Consider a diagnosis of **ovarian cancer** in women having any of the following symptoms persistently or frequently – particularly if **more than 12 times per month** (especially if she is **≥ 50 yrs**):

- ≥ 50 yrs and has had symptoms within the last 12 months that suggest Irritable Bowel Syndrome (IBS)
- Red flag symptoms (e.g. unexplained weight loss, fatigue, change in bowel habit)
- Persistent abdominal distension (“bloating”)
- Feeling full (early satiety) and/or loss of appetite
- Pelvic or abdominal pain
- Increased urinary urgency and/or frequency

Perform abdominal and pelvic examination

(NB: NICE recommends examination but acknowledges limitations in its detection of adnexal pathology)

Pelvic or abdominal mass or ascites on examination

Urgent (suspected cancer) gynaecology referral

(If possible refer with CA125 result and simultaneously request urgent pelvic USS)

No mass or ascites on examination

Measure serum CA125

≥ 35 kU/L

Increased significance if ≥50yrs

Request “Investigation of suspected ovarian cancer USS” using SCI Gateway proforma

USS suspicious of ovarian cancer

Urgent gynaecology referral automatically generated by radiology.

USS NOT suspicious of ovarian cancer

Calculate RMI1

RMI ≥250 - Urgent (suspected cancer) gynaecology referral

RMI < 250 - Repeat CA125 at 4 weeks. Then, if CA125 remains ≥ 35 kU/L for urgent gynaecology referral

< 35 kU/L

Reassess, investigate and refer appropriately as symptoms dictate.

Have a low threshold for a routine pelvic USS as well as considering other systems.

30% women with ovarian cancer have a normal CA125.

Risk of Malignancy Index 1 (RMI1)

$RMI1 = U \times M \times CA125$

U = ultrasound score (0,1,3)

M = Menopausal status (1 = pre, 3 = post)

CA125 level